

 Announcement



Kindly note: We're currently experiencing a high volume of inquiries and calls, potentially leading to delays in our initial response.

Dermatology 23S Module 2

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Day 1 Scenario 1

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Day 1 Scenario 1

by [Derek Kayanja \(Tutor\)](#) - Sunday, 12 November 2023, 3:06 PM

Hi Group,

Welcome to week two.

Here is our first scenario

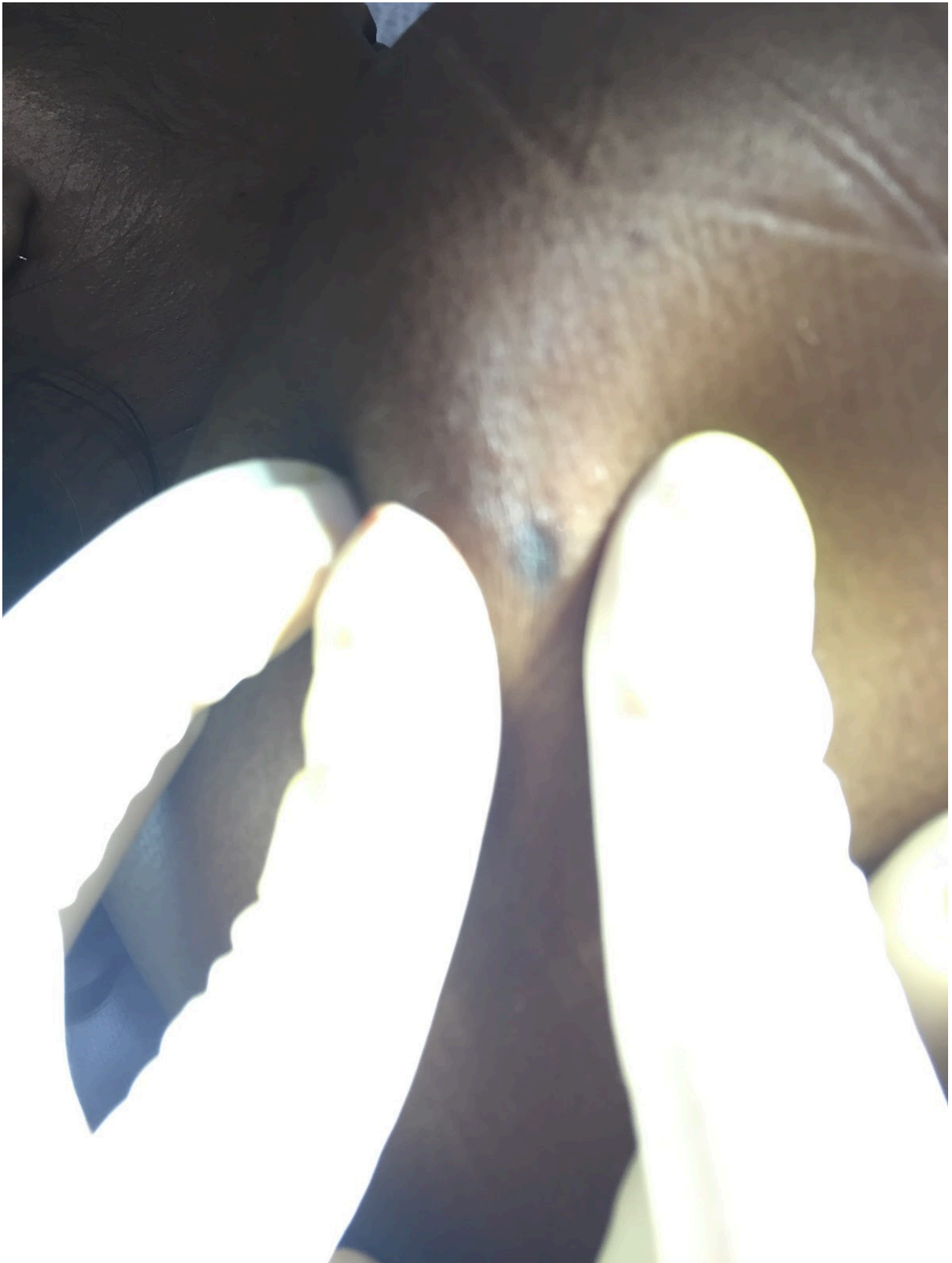
A 50-year-old Asian female presented with a firm, pigmented, dome shaped papule of about 5 mm in diameter over the left arm (see image below).



She reported an increase in the size of the lesion over 5 years from a speck to attainment of the present dimensions. There is occasional itching.

Please answer all the questions below:

- What is the most likely cause?
- Pinching on either side causes a depression (see the following image):



- What is the name of this depression?
- What immunohistochemical stain is likely to be positive?

100 words

Reply



Re: Day 1 Scenario 1

by [Zarmeena Khan](#) - Monday, 13 November 2023, 7:54 AM

Thankyou for the case Dr Derek,
-What is the most likely cause?
The lesion is typical for dermatofibroma,

These are also known as benign fibrous histiocytoma, benign skin lesion that typically presents as a small, firm nodule on the skin. They often manifest as reddish-brown or purplish growths and are most commonly found on the lower extremities, particularly the legs, but can appear anywhere on the body. They are more frequent in women and usually affect adults.

-What is the name of this depression?

Pinching on either side causes a depression,

This is a characteristic dimple or "dimple sign" when pinched. They are often asymptomatic but may sometimes be itchy, painful, or tender, particularly if irritated or traumatized.

-What immunohistochemical stain is likely to be positive?

Immunohistochemical staining in dermatofibroma typically shows positive staining for Factor XIIIa. Factor XIIIa is a marker that highlights dermal dendrocytes, which are commonly found in dermatofibromas. This marker aids in distinguishing dermatofibromas from other skin lesions.

Factor XIIIa (a transglutaminase present in many cells including dermal dendrocytes) positivity is a characteristic feature but it's important to note that the diagnosis of dermatofibroma is primarily clinical, and immunohistochemistry is typically used to support the diagnosis rather than being a primary diagnostic tool.

References :

1. Zalaudek, I., & Argenziano, G. (2018). Dermoscopy of Dermatofibroma. Clinical and Experimental Dermatology, 43(4), 394–395. <https://doi.org/10.1111/ced.13343>

2. Zaballos, P., Puig, S., Llambrich, A., & Malvehy, J. (2005). Dermoscopy of Dermatofibromas: A Prospective Morphological Study of 412 Cases. Archives of Dermatology, 141(4), 536. <https://doi.org/10.1001/archderm.141.4.536>

3. Requena, L., & Sanguenza, O. P. (1995). Benign Fibrous Histiocytoma/Dermatofibroma. A Subclassification. Journal of Cutaneous Pathology, 22(3), 194–202. <https://doi.org/10.1111/j.1600-0560.1995.tb00771.x>

4. Prieto, V.G., Reed, J.A. and Shea, C.R. (1995). Immunohistochemistry of dermatofibromas and benign fibrous histiocytomas. Journal of Cutaneous Pathology, [online] 22(4), pp.336–341. doi:<https://doi.org/10.1111/j.1600-0560.1995.tb01416.x>.

296 words

Reply



Re: Day 1 Scenario 1

by [Derek Kayanja \(Tutor\)](#) - Tuesday, 14 November 2023, 1:10 PM

Thank you Zarmeena, that is the correct diagnosis, good post.

10 words

Reply



Re: Day 1 Scenario 1

by [Xiao Liey Ding](#) - Monday, 13 November 2023, 8:35 AM

1. What is the most likely cause?

Dermatofibromas

- usually present as small, firm nodules on the skin.
- commonly appears on the extremities, particularly the lower legs.
- It is a type of fibrous tumor that arises in the dermis, the second layer of the skin.

This case also appear in our week1 scenario 1 and there are numerous sub-variant with cellular type show tendency of malignant.

2. What is the name of this depression?

"dimple sign" or "buttonhole sign" .

3. What immunohistochemical stain is likely to be positive?

Dermatofibromas typically show positivity for Factor XIIIa on immunohistochemical staining (Cheng, H. and Emanuel, P., 2013). Factor XIIIa is an enzyme that is involved in blood coagulation and is commonly used as a marker for cells of histiocytic origin, including dermal dendrocytes. In dermatofibromas, Factor XIIIa staining is often used to highlight the spindle-shaped cells within the lesion. This immunohistochemical stain helps in distinguishing dermatofibromas from other skin lesions and tumors.

Reference:

Cheng, H. and Emanuel, P. (2013). *Dermatofibroma pathology*/ *DermNet NZ*. [online]. Available at: <https://dermnetnz.org/topics/dermatofibroma-fibrous-histiocytoma-pathology> (Accessed on 13 Nov 2023).

182 words

Reply



Re: Day 1 Scenario 1

by [Derek Kayanja \(Tutor\)](#) - Tuesday, 14 November 2023, 1:11 PM

Thank you Xiao Liey, good post.

6 words

Reply



Re: Day 1 Scenario 1

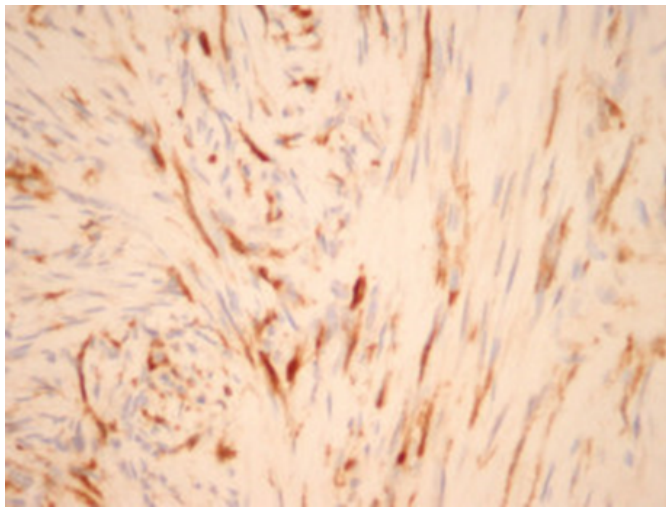
by [Pooran Outar](#) - Monday, 13 November 2023, 1:56 PM

Diagnosis: Dermatofibroma.

Clinical Sign: Dimple Sign

Immunohistological staining is positive for Factor XIIIa (Cheng and Emanuel, 2013).

Question: Is anyone able to comment as to why this occurs?



We have covered Dermatofibromas in previous discussions and everyone has already answered the questions brilliantly, so I would like to summarize the histopathological characteristics for each variant (Cheng and Emanuel, 2013); for anyone interested in more detailed reading.

Dermatofibroma Variant	Histopathological Characteristics
Typical Dermatofibroma	Poorly defined proliferation of fibrohistiocytic cells within the dermis. Overlying grenz zone of sparing. Peripheral entrapment of collagen. Acanthotic epidermis with increased basal layer pigmentation. Basaloid induction of the epidermis in some cases.
Cellular Dermatofibroma	Increased cellularity with a swirling, storiform pattern. Less prominent peripheral entrapment of collagen. Higher chance of recurrence and reported metastasis. Possible central necrosis. CD34 may show focal positivity.
Epithelioid Dermatofibroma	Centrally located, circumscribed tumour with an epidermal collarette. Cells arranged in sheets with epithelioid morphology. Abundant eosinophilic cytoplasm, round vesicular nuclei, and prominent nucleoli. May resemble other epithelioid tumours. ALK-1 translocation is common.

Aneurysmal Dermatofibroma	Rapidly growing blue-brown nodule with periods of rapid growth due to hemorrhage. Cleft-like hemorrhagic spaces lacking an endothelial lining. Fairly cellular tumor with haemosiderin deposition. High recurrence rate, reported lymph node involvement.
Atypical Dermatofibroma	Focal areas with large polymorphous cells ('monster cells'). Rare reports of metastasis and death. Aggressive behavior, though some cases may now be reclassified as sarcomas.
Haemosiderotic Dermatofibroma	Extensive haemosiderin deposition. Clinically pigmented, may mimic melanoma.
Lipidised Dermatofibroma	Foamy histiocytes predominate with marked hyalinization of collagen.
Fibrocollagenous Dermatofibroma	Prominent storiform or swirled pattern. More common in disseminated dermatofibroma in immunosuppressed patients.
Clear Cell Dermatofibroma	Atypical morphology. Sheets of cells within the dermis, occasionally extending to the subcutis. Cells have vesicular nuclei without significant atypia.
Deep Fibrous Histiocytoma	Resembles dermatofibroma histologically but affects deep subcutis or soft tissues. Diffuse storiform pattern and haemangiopericytoma-like changes. Up to one in five cases may recur, with reported cases of metastasis and death.
Palisading Dermatofibroma	Characterized by prominent palisading of nuclei. Mimics Verocay bodies seen in schwannoma. Lack of capsule and negative S100 staining differentiate it from schwannoma.

Reference list:

Cheng, H. and Emanuel, P. (2013). *Dermatofibroma (fibrous histiocytoma) pathology* | DermNet. [online] dermnetnz.org. Available at: <https://dermnetnz.org/topics/dermatofibroma-fibrous-histiocytoma-pathology>.

[Reply](#)**Re: Day 1 Scenario 1**by [Jillian Simpson](#) - Monday, 13 November 2023, 5:53 PM

The most likely cause is a dermatofibroma often caused by an insect bite or minor trauma (Morris-Jones, 2019). They are more common in females than males and vary in size from 0.5-1.5 cm, although most are 7-10 mm diameter (Oakley, 2020). The colour may be pink to light brown in white skin, and dark brown to black in dark skin (Oakley, 2020) as shown in the case above. They do not usually cause symptoms but are sometimes painful, tender or itchy, again as in the case above.

The name given to a depression caused by the pinch test is a dimple.

Factor XIIIa is a classic immunohistochemical marker for the diagnosis of a dermatofibroma (Llamas-Velasco et al, 2022).

With regards to your question Pooran as to why they stain, I think it's due to the dermatofibroma containing dermal dendrocytes that contain Factor XIIIa which then show a positive stain, but I am struggling to fully understand the histology side of things.

References:

Morris-Jones, R. (2019). ABC of Dermatology. Seventh Edition. Wiley Blackwell. P187-188.

Oakley, A (2020). Dermatofibroma (histiocytoma) | DermNet NZ. [online] Available at:

<https://dermnetnz.org/topics/dermatofibroma> (Accessed 13/11/23).

Llamas-Velasco M, Mentzel T, Ovejero-Merino E, Teresa Fernández-Figueras M, Kutzner H. CD64 Staining in Dermatofibroma: A Sensitive Marker Raising the Question of the Cell Differentiation Lineage of This Neoplasm. Journal of Molecular Pathology. 2022; 3(4):190-195. <https://doi.org/10.3390/jmp3040016>

223 words

[Reply](#)**Re: Day 1 Scenario 1**by [Pooran Outar](#) - Monday, 13 November 2023, 8:06 PM

Hi Jillian,

Thank you for your response. I myself find the histopathology aspects a bit confusing.

Kind regards,
Pooran Outar
colleague

21 words

[Reply](#)**Re: Day 1 Scenario 1**by [Sophie Holloran](#) - Monday, 13 November 2023, 8:43 PM

W2D1

I agree with everyone's differential, that this is most likely to be a dermatofibroma. This is confirmed clinically by the 'dimple sign'. This is a clinical diagnosis made on clinical examination along with key features in the history.

I found out the following information about immunohistological staining in dermatofibroma:

As discussed last week, it is important to be able to differentiate benign dermatofibromas from its malignant differential – dermatofibrosarcoma protuberans (DFSP). Benign dermatofibromas will stain positively for factor XIIIa, D2-40 and negatively for CD34. DFSP will stain positive for Ki-67 and CD34 and negatively for factor XIIIa.

I found this really good table on Up to Date which summarises the histological staining differences for spindle cell dermal tumours:

Summary of immunohistochemical staining of dermatofibrosarcoma protuberans (DFSP) and other spindle cell dermal tumors

	CD34	XIIIa	S-100	SMA	Desmin	Vimentin	CD44	Hyaluronate
DFSP	+	-	-	-	-	+	+/-	+
Dermatofibroma	-	+	-	+	-	+	+	+/-*
Nodular fasciitis	-	-	-	+	-	-		
Nerve sheath tumors	+/-	-	+	-	-	-		
Fibrosarcoma	-	-	-	+	-	+		
Malignant fibrous histiocytoma	+/-	-	-	-	+/-	+		

SMA: smooth muscle actin; +: more than 80 percent of cases staining; -: less than 20 percent of cases staining; +/-: variable staining.

* For dermatofibromas, cells stain intensely with CD44 while the stroma stains faintly for hyaluronate; DFSPs are strongly positive for hyaluronate while CD44 staining is diminished or absent.

Derived from: Haycox CL, et al. *J Am Acad Dermatol* 1997; 37:438 and Calikoglu E, et al. *J Cutan Pathol* 2003; 30:185.

Graphic 73370 Version 3.0

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From a practical perspective, I think that if there was clinical concern about a lesion in the history or with concerning features on dermoscopy, it would be listed for urgent excision to exclude a malignant lesion. I think that the histological staining is really useful to differentiate which specific type of spindle cell tumour has been excised, as I would imagine this is likely to have an impact on excision margins or further treatment for example. This is beyond my current understanding - does anyone else have anything to add about further management of spindle cell tumours once they have been excised?

References:

1. Zelman et al. (2022) *Dermatofibroma – Cutaneous fibrous histiocytoma*. Pathology Outlines.com. Available at: <https://www.pathologyoutlines.com/topic/softtissuebfh.html> (Accessed 13/11/23)
2. Myers, Fillman (2022) *Dermatofibroma*. StatPearls. Available at: <https://www.ncbi.nlm.nih.gov/books/NBK470538/#:~:text=Benign%20dermatofibromas%20will%20stair> (Accessed 13/11/23)
3. Yu (2023) *Dermatofibroma (benign fibrous histiocytoma)*. Up to Date. Available at: https://www.uptodate.com/contents/dermatofibroma-benign-fibrous-histiocytoma?search=dermatofibroma&source=search_result&selectedTitle=2~25&usage_type=default&display_rank= (Accessed 13/11/23)

263 words

Reply



Re: Day 1 Scenario 1

by [Xiao Liey Ding](#) - Tuesday, 14 November 2023, 8:10 AM

Hi Sophie,

I like the summary of immunohistochemical staining! And I think we all agree that different type of spindle cell tumour will have an impact on excision margins and further treatment.

The histopathological evaluation of excised spindle cell tumours includes an assessment of surgical margins. It is crucial to ensure that the tumour has been completely removed and clear margins reduce the risk of local recurrence and rarely, metastasis.

In certain cases, such as Dermatofibrosarcoma protuberans, adjuvant therapies such as radiation therapy or systemic treatments may be considered after Mohs micrographic surgery or wide local excision (Brooks and Ramsey, 2023). The decision for adjuvant therapy is often based on the specific characteristics of the tumour and the individual patient's risk profile. Patients may undergo regular follow-up examinations to monitor for any signs of recurrence, especially the first 3 years after treatment. Follow-up care may involve clinical examinations, imaging studies, or other diagnostic tests.

Histopathological evaluation is also helpful in determining clinical trials, especially for rare or aggressive spindle cell tumours. Clinical trials explore new treatment strategies and contribute to advancements in the field.

Reference:

Brooks, J. and Ramsey, M.L. (2023) . Dermatofibrosarcoma Protuberans| National Library of Medicine. *StatPearls [online]*. Available at: <https://www.ncbi.nlm.nih.gov/books/NBK513305/> (Accessed 14 Nov 2023)

207 words

Reply



Re: Day 1 Scenario 1

by [Derek Kyanja \(Tutor\)](#) - Tuesday, 14 November 2023, 1:13 PM

Thank you Sophie, this is an interesting and useful post.

10 words

Reply



Re: Day 1 Scenario 1

by [Derek Kyanja \(Tutor\)](#) - Tuesday, 14 November 2023, 1:12 PM

Thank you Jillian good post.

5 words

Reply



Re: Day 1 Scenario 1

by [Derek Kyanja \(Tutor\)](#) - Tuesday, 14 November 2023, 1:11 PM

Thank you Pooran, very good post nice table.

8 words

Reply



Re: Day 1 Scenario 1

by [Derek Kyanja \(Tutor\)](#) - Tuesday, 14 November 2023, 1:18 PM

Hi Group,

Thanks to everybody who has responded to our first scenario, you have all got the correct diagnosis and I am sure that you shall all be able to recognise a dermatofibroma when one presents.

Dermatofibroma, are benign tumours of the skin presenting as a firm hyperpigmented dome-shaped papule, which may be tan to pink in light-skinned individuals, with a rim of hyperpigmentation.

Lateral compression causes dimpling, known as the Dimple sign or the Fitzpatrick's sign, due to tethering of the dermis.

Other interesting signs are:

The Darier sign is seen in urticaria pigmentosa (cutaneous mastocytosis).

Hutchinson's sign is elicited in subungual melanoma, presenting as melanonychia with pigmentation of the proximal nail fold.

Albright's sign is the characteristic dimple over the knuckle in hereditary osteodystrophy.

Leser-Trelat sign is the sudden appearance of multiple seborrheic keratosis, considered a paraneoplastic syndrome most commonly with gastrointestinal malignancy.

Acquired melanocytic naevi are soft, round to oval macules or papules. Nodular BCC, the commonest subtype, is a smooth pearly papule/nodule with overlying telangiectasia. Nodular melanoma presents as a blue-black or pinkish-red papule or plaque that may ulcerate or bleed.

Seborrheic keratosis are tan to brown or deeply pigmented with a "waxy", stuck-on appearance.

Dermatofibroma stains positive for Factor XIIIa. Dermatofibrosarcoma protuberans is the malignant transformation of the dermatofibroma, which stains negative for Factor XIIIa and is CD34 positive. BerEP4 positive is characteristic of BCC, S100 for melanocytic lesions, Cdx1 for Langerhans cells, CK20 for epithelial cells and Merkel cells.

245 words

Reply



Re: Day 1 Scenario 1

by [Sandeep Kumar](#) - Wednesday, 15 November 2023, 1:09 PM

I would agree with others- this looks like a dermatofibroma/ histiocytoma.

Diagnosis is clinical and diagnostic investigations are rarely needed.

Same goes with the treatment, no treatment needed as they are benign and rarely cause any problems or discomfort.

Many people go for aesthetic removal which is not clinically indicated otherwise.

The immunohistochemical stain which comes positive in cases of dermatofibroma classically is - Factor XIIIa staining the dendrocytes.

The depression is termed as Pinch sign. Acknowledging the characteristic of dermatofibroma not attached to the overlaying skin.

I would like to bring the cohort's attention to an alternative staining method - ERG Immunostaining using utility of erythroblast transformation-specific-related gene.

Although I do not feel any diagnostic utility should be used as a diagnostic tool for dermatofibroma unless the presentation is very atypical or clinical suspicion.

References:

dermnetnz.org. (n.d.). Dermatofibroma (histiocytoma) | DermNet NZ. [online] Available at: <https://dermnetnz.org/topics/dermatofibroma>. [Accessed 14/11/2023]

Prieto, V.G., Reed, J.A. and Shea, C.R. (1995). Immunohistochemistry of dermatofibromas and benign fibrous histiocytomas. Journal of Cutaneous Pathology, [online] 22(4), pp.336-341. doi:<https://doi.org/10.1111/j.1600-0560.1995.tb01416.x>.

Yamada Y, Ichiki T, Susuki Y, et al Diagnostic utility of ERG immunostaining in dermatofibroma Journal of Clinical Pathology 2023;76:536-540.

190 words

Reply

**Re: Day 1 Scenario 1**by [Derek Kayanja \(Tutor\)](#) - Sunday, 19 November 2023, 8:54 AM

Sandeep thank you good post.

5 words

Reply

**Re: Day 1 Scenario 1**by [Siddharth Saluja](#) - Friday, 17 November 2023, 10:50 AM

1) This looks typical of a dermatofibroma. Benign fibrous histiocytomas of the skin, superficial/cutaneous benign fibrous histiocytomas, and common fibrous histiocytomas are other names for dermatofibromas. Rather than precisely characterizing the cellular lineage, the term "fibrous histiocytoma" more accurately describes the morphologic appearance of the cell populations that make up these lesions. Patients with dermatofibromas frequently have a history of potentially initiating local trauma at the site, such as a thorn or wood splinter bite or superficial puncture wound. All ages are affected by dermatofibromas, but the 20s to 40s are the most prevalent time for them to appear. Females are more likely than males to acquire dermatofibromas; some reports even suggest a 2:1 female to male ratio. (1)

2) Dermatofibroma present with the dimple sign which is the same as described above.

3) Factor XIIIa staining is typically positive in dermatofibroma when analyzed through immunohistochemical techniques. Factor XIIIa is a substance that identifies dermal dendrocytes, which are frequently present in dermatofibromas. The use of this marker helps in differentiating dermatofibromas from other abnormalities on the skin.

The presence of Factor XIIIa, a transglutaminase found in various cells like dermal dendrocytes, is a distinctive characteristic. However, it is crucial to understand that the diagnosis of dermatofibroma relies mainly on clinical observations. Immunohistochemistry is usually utilized as a means of confirming the diagnosis, rather than being the main diagnostic tool. (2)

References

1)Myers, D.J. and Fillman, E.P. (2023). Dermatofibroma. [online] PubMed. Available at:

<https://www.ncbi.nlm.nih.gov/books/NBK470538/#:~:text=Dermatofibroma%20is%20a%20commonly%20occu>

2)dermnetnz.org. (n.d.). Dermatofibroma (histiocytoma) | DermNet NZ. [online] Available at:

<https://dermnetnz.org/topics/dermatofibroma>.

254 words

Reply

**Re: Day 1 Scenario 1**by [Derek Kayanja \(Tutor\)](#) - Sunday, 19 November 2023, 8:54 AM

Thank you Siddharth, good post.

5 words

Reply

**Re: Day 1 Scenario 1**by [Sadaf Basharat](#) - Sunday, 19 November 2023, 4:50 PM**1)Most likely cause is Dermatofibroma.**

Dermatofibromas, is also called as benign fibrous histiocytomas, are common benign skin lesions that usually appear small, firm nodules over the skin. They often have a reddish-brown or purplish color and are usually present on the lower extremities, although they can occur anywhere on the body. Dermatofibromas are more

common in women and tend to affect adults. Treatment for dermatofibromas is typically not necessary unless they cause discomfort or cosmetic concerns. However, if desired, treatment options can include surgical removal, cryotherapy (freezing), or laser therapy.

2) The depression shown in the picture is usually called as a "Dimple Sign".

The "dimple sign" in dermatofibroma refers to a characteristic feature of this skin lesion. When you pinch the sides of a dermatofibroma, it often causes a dimpling or depression in the center. This is a diagnostic sign that can help distinguish dermatofibromas from other skin conditions.

3) Dermatofibromas in immunohistochemical staining commonly exhibit positivity for Factor XIIIa and CD68.

References:

1. Myers, D.J. and Fillman, E.P., 2017. Dermatofibroma.
2. Zaballos, P., Puig, S., Llambrich, A., & Malvehy, J. (2005). Dermoscopy of Dermatofibromas: A Prospective Morphological Study of 412 Cases. Archives of Dermatology, 141(4), 536. <https://doi.org/10.1001/archderm.141.4.536>
3. Cheng, H. and Emanuel, P. (2013). *Dermatofibroma pathology* | DermNet NZ. [online]. Available at: <https://dermnetnz.org/topics/dermatofibroma-fibrous-histiocytoma-pathology> (Accessed on 13 Nov 2023).

216 words

Reply



Re: Day 1 Scenario 1

by [Bhavika Patel](#) - Monday, 20 November 2023, 9:13 PM

Dermatofibroma as mentioned above, are typically asymptomatic benign firm nodular dermal lesions caused by "proliferations of fibroblasts and histiocytes" (Schechter, 2020). Commonly appears on sites of minor trauma or insect bites on the extremities. Multiple lesions can be seen on immunosuppressed patients. Typical dermatofibromas can self-resolve without treatment as the skin cells turnover, however in cases of severe itching or pain, surgical excision or cryotherapy can be considered for treatment.

Variants of dermatofibromas include (Oakley, 2020. Cheng, 2013):

1. Cellular – accounts for 5-10% of all dermatofibromas.
2. Atypical – caution for underlying undiagnosed malignant changes.
3. Epithelioid – differentials include amelanotic melanoma, epithelioid sarcoma.
4. Aneurysmal – "rapidly growing blue-brown nodule"
5. Cholesterotic
6. Palisading
7. Lipidized ankle-type

Pinching will create a 'Dimple Sign' also known as a 'Buttonhole Sign' characterised by a central depression on pinching the lesion.

Immunohistochemical staining would be positive for Factor XIIIa for dermatofibromas, however negative Factor XIIIa staining in their malignant counterparts, dermatofibrosarcoma protuberans, which instead stains positive for CD34.

Oakley, A (2020). Dermatofibroma (histiocytoma). DermNet NZ. [Online]. Available at: <https://dermnetnz.org/topics/dermatofibroma> [Accessed 19th November 2023]

Cheng, H (2013). Dermatofibroma pathology. DermNet NZ. [Online]. Available at: <https://dermnetnz.org/topics/dermatofibroma-fibrous-histiocytoma-pathology> [Accessed 19th November 2023]

Schechter, S.A., Bresler, S.C. and Patel, R.M. (2020). Fat necrosis with an associated lymphocytic infiltrate represents a histopathologic clue that distinguishes cellular dermatofibroma from dermatofibrosarcoma protuberans. Journal of Cutaneous Pathology, 47(10), pp.913-916.

225 words

Reply

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